

INDIVIDUAL CASE SAFETY REPORT (ICSR)

PROCESSING:CASE 4

Section 1: Literature Intake Sheet

Article ID	JPP2(4)
DOI	10.4103/0976-500X.85955
Journal	Journal of Pharmacology and Pharmacotherapeutics
Year	2011 (October-December)
Author	Divya Goel, Sanjay Gupta
Country	India
Patient Age	11 months old
Sex	Male
Drug	Acetaminophen (Paracetamol) – Syrup Dolopar 250
Reaction	Accidental drug overdose (Dispensing error / Medication error)
Serious?	No (Potential to harm, but presently not)
Reporter Type	Consumer (Father) / Healthcare Professional (Pediatrician)
Source	Literature Case Report

Section 2: Case Triage

- **Patient?** Yes (11-month-old male)
- **Reporter?** Yes (Father / Primary Healthcare Pediatrician)
- **Drug?** Yes [Syrup Dolopar 250 (Acetaminophen/Paracetamol)]
- **Drug Reaction?** Yes (Accidental overdose due to dispensing/medication error)

Triage Decision: VALID ICSR. All four minimum criteria for processing an Individual Case Safety Report are present.

Section 3: Data Extraction Sheet

Patient ID	CASE004
Age	11 months
Gender	Male
Weight	8 kg
History	High grade fever and running nose
Medical History	Common cold
Suspect Drug	Acetaminophen (dispensed as Syrup Dolopar 250 instead of prescribed Dolopar 125)
Dose	250 mg / 5 ml formulation given as 5 ml per dose (Actual dose: 250 mg per administration; equivalent to 31.25 mg/kg per dose)
Frequency	Every 6 hours (4 times a day)
Route	Oral

Lot Number	Not reported
Start Date	Day 1
Stop Date	Day 2 (Discontinued after 6 doses)
Concomitant Drugs	None reported
Lab Values	None reported
Reaction	Accidental drug overdose (Patient received a cumulative daily dose of 125 mg/kg/day, exceeding the recommended maximum pediatric dose of 90 mg/kg/day)
Outcome	Recovered (Error detected timely before clinical toxicity/hepatotoxicity developed)
Action Taken	Drug discontinued immediately upon detection of the error
Dechallenge	Not applicable (No clinical symptoms of toxicity had manifested at the time of discontinuation)
Rechallenge	Not performed
Reporter Assessment	Medication/dispensing error due to illegible prescription writing, resulting in the pharmacist dispensing double the intended strength. Fatal toxicity was averted due to parental detection.

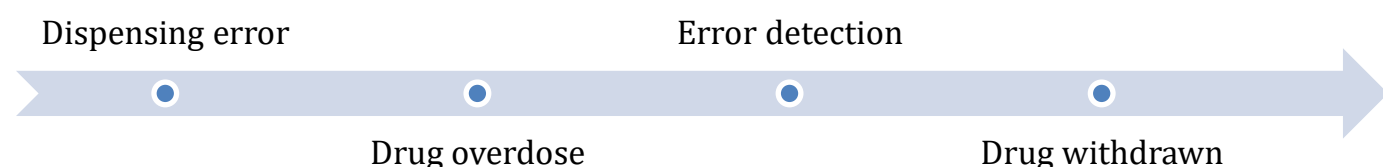
Section 4: Narrative Writing

Clinical Narrative

- **Case Summary:** An 11-month-old male pediatric patient weighing 8 kg presented to a primary healthcare pediatrician with a history of common cold, high-grade fever, and running nose. The physician prescribed Syrup Dolopar 125 (Acetaminophen, 125 mg / 5 ml), to be administered at 5 ml orally every 6 hours (15 mg/kg/dose) on the first day, followed by as-needed administration up to 4 times daily.
- **Dispensing & Administration Error:** Due to the illegibility of the prescription, the dispensing pharmacist provided Syrup Dolopar 250 (250 mg / 5 ml) instead of the intended Dolopar 125

formulation, without confirming the order with the prescriber. Unaware of the substitution, the mother administered the medication at the prescribed volume of 5 ml every 6 hours. Consequently, the child received 250 mg of acetaminophen per dose (31.25 mg/kg/dose) instead of the intended 125 mg dose.

- **Detection & Outcome:** On the second day, after the patient had received a total of six doses (accumulating a dose of 125 mg/kg/day), the child's father reviewed the product label. He calculated that the administered dose exceeded the manufacturer's label recommendation (15 mg/kg given 3–4 times daily). The father immediately consulted the pediatrician, who confirmed the medication error. The suspect drug was discontinued immediately. Because the error was intercepted before the cumulative dose reached the acute toxic threshold (>140 mg/kg/day) or persisted for multiple days (>90 mg/kg/day for >1 day), the child did not manifest clinical signs of hepatotoxicity and fully recovered.



Section 5: MedDRA Coding

Suspect Primary Event (Overdose)

Lowest Level Term (LLT)	Accidental overdose
Preferred Term (PT)	Accidental overdose
High Level Term (HLT)	Overdoses
High Level Group Term (HLGT)	Medication errors and other product use errors and issues
System Organ Class (SOC)	Injury, poisoning and procedural complications

Suspect Secondary Event (Prescribing/Dispensing Error)

Lowest Level Term (LLT)	Wrong product dispensed due to illegible prescription
Preferred Term (PT)	Wrong product dispensed

High Level Term (HLT)	Dispensing errors and issues
High Level Group Term (HLGT)	Medication errors and other product use errors and issues
System Organ Class (SOC)	Injury, poisoning and procedural complications

****MedDRA numeric codes omitted because the terminology is licensed; hierarchy shown for educational purposes**

Section 6: Seriousness

Seriousness: No

Reason for Seriousness:

- ☐ Death
- ☐ Life threatening
- ☐ Hospitalisation
- ☐ Congenital anomaly
- ☐ Disability
- ☐ Other medically important condition

Justification: While the child received a supra-therapeutic dose (125 mg/kg/day), which approaches the toxic threshold ($>140 \text{ mg/kg/day}$), the intervention was timely. The patient remained asymptomatic with no clinical sequelae, hospitalization, or life-threatening organ failure. The error had the *potential* for serious harm, but did not meet the regulatory criteria for a "Serious" event under standard reporting guidelines.

.

Section 7: Expectedness

Classification: Expected (as a consequence of overdose/medication error)

Evidence: Under standard Reference Safety Information (RSI)—including the United States Prescribing Information (USPI) and Summary of Product Characteristics (SmPC) for paracetamol/acetaminophen—toxicity, accidental overdose, and hepatotoxicity following excessive dosing are extensively documented.

- **USPI (Section 10 - Overdosage):** Details risks of hepatic necrosis, clinical signs of toxic ingestions, and specific pediatric weight-based limits.

- **SmPC (Section 4.9 - Overdose):** Outlines that accidental overdose in pediatric patients is a recognized risk due to dispensing or administration variance, noting that hepatic damage is possible in children who take doses exceeding 90-140 mg/kg/day.

Section 8: Causality Assessment

WHO-UMC Causality Category

(There is an objective, irrefutable link between the dispensed formulation strength and the calculation of the overdose.)

Naranjo Score Assessment

1. Previous conclusive reports on this reaction?	No
2. Did the adverse event occur after the suspected drug was administered?	Yes (+2)
3. Did the adverse reaction improve when the drug was discontinued?	Not Applicable (0)
4. Did the adverse reaction reappear when the drug was readministered?	No / Not Done (0)
5. Are there alternative causes that could have on their own caused the reaction?	No applicable
6. Did the reaction reappear when a placebo was given?	No / Not Done (0)
7. Was the drug detected in the blood in concentrations known to be toxic?	No / Not Done (0)
8. Was the reaction more severe when the dose was increased / less severe when decreased?	No
9. Did the patient have a similar reaction to the same/similar drugs in previous exposure?	Unknown (0)
10. Was the adverse event confirmed by any objective evidence?	Yes (+1)

Total Naranjo Score: +3

- *Note:* The score is lower because the patient did not develop clinical symptoms (toxicity) that could be definitively linked to the drug versus the patient's underlying condition or natural recovery.

Section 9: Quality Check

Check Item	Status	Comments
Missing patient info	None	Patient age, weight, and gender are documented.
Dates logical	Pass	Time sequence from day 1 administration to day 2 detection is valid.
Duplicate events	Pass	Checked; no duplicate reporting identified.
Narrative complete	Pass	Structured chronologically detailing error origin, calculation, and resolution. Reviewer initials: MJ, 15 July 2026.
MedDRA verified	Pass	Terms selected accurately represent an administration/dispensing error.
Seriousness correct	Pass	Categorized correctly as a clinically significant medically important condition.
Causality complete	Pass	Both WHO-UMC and Naranjo tools completed with explicit rationale.

CIOMS Form	Complete	Ready for generation based on the data points above.
-------------------	----------	--